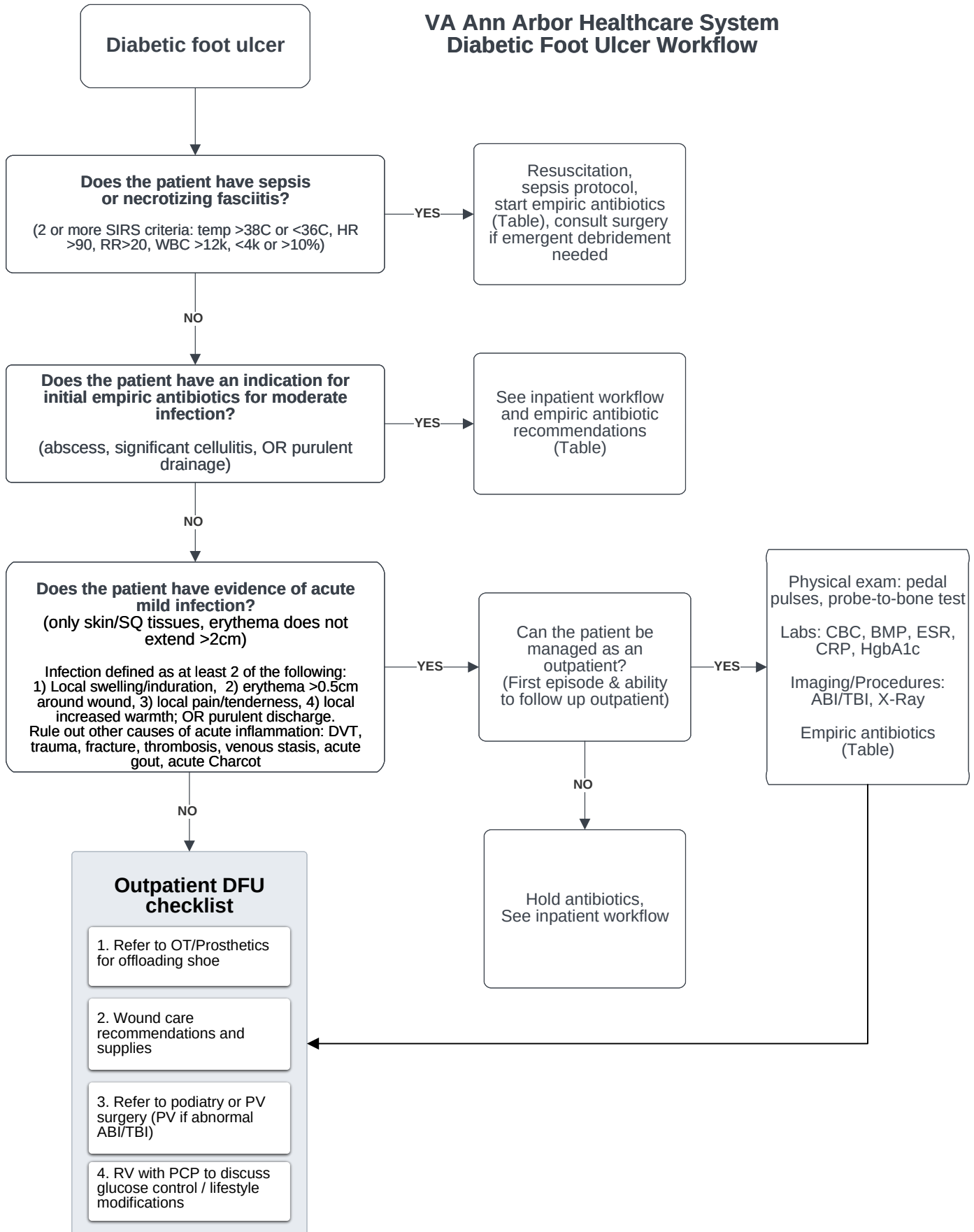


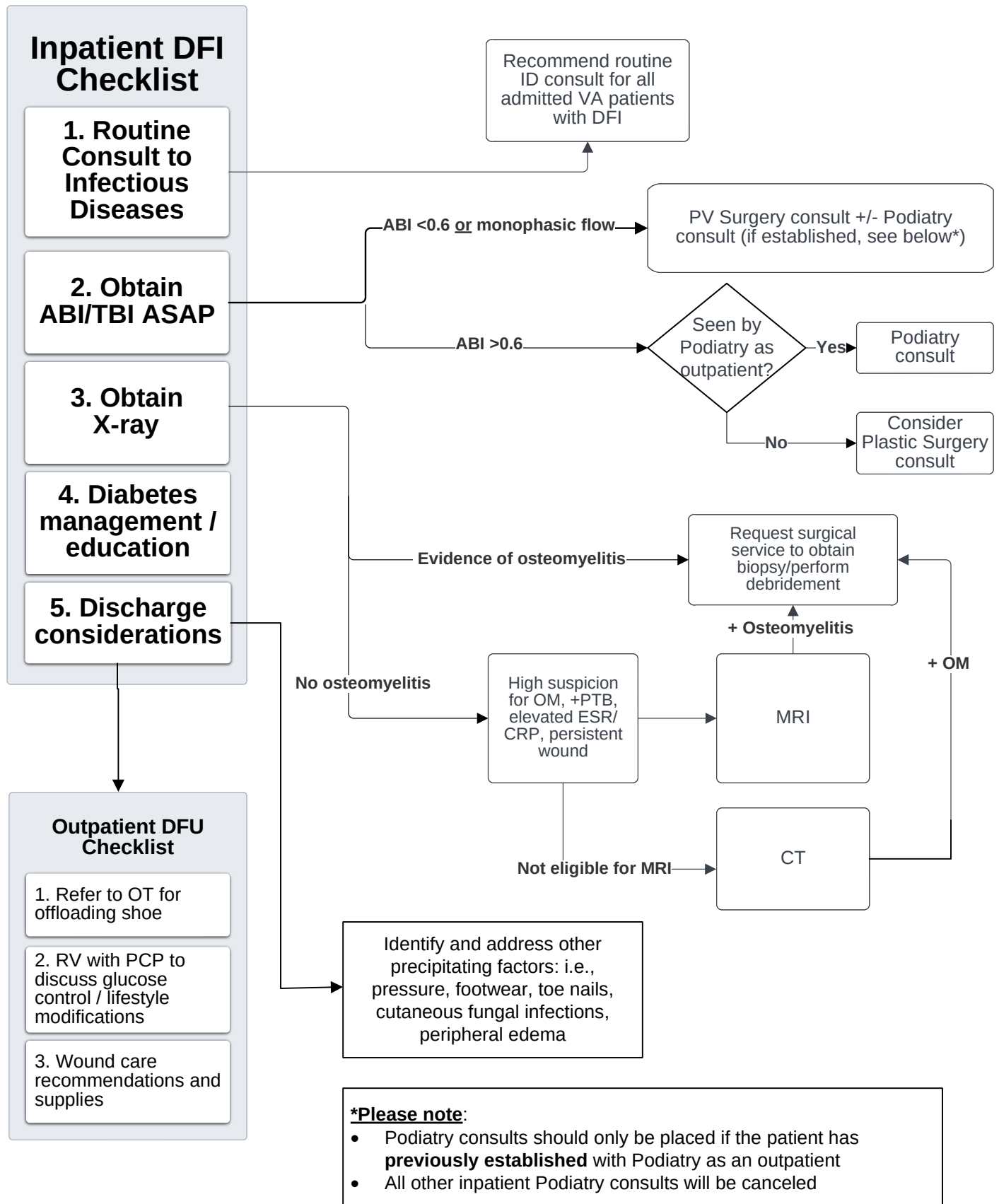
VA Ann Arbor Healthcare System Diabetic Foot Ulcer Workflow



ABI/TBI: ankle-brachial index / toe brachial index; DVT: deep vein thrombosis; OM: osteomyelitis; OT: occupational therapy; PCP: primary care provider; PTB: probe-to-bone; PV: vascular surgery; RV: return visit; SIRS: systemic inflammatory response syndrome; SQ: subcutaneous

VA Ann Arbor Healthcare System

Workflow for Inpatients with Diabetic Foot Infection



VA Ann Arbor Healthcare System

Diabetic Foot Infection Classification and Empiric Antibiotic Guidance

Preferred oral antibiotics for mild Diabetic Foot Infection (DFI)	
Most common organisms involved are Streptococcus species and methicillin-sensitive Staphylococcus aureus	
No history of MRSA	Cephalexin 1g PO q8h OR Amoxicillin/clavulanic acid 875/125 mg PO q8h-12h OR Cefuroxime 500-1000 mg PO q8h or q12h (penicillin allergy)
History of MRSA or purulent infection with no culture results	Doxycycline 100 mg po BID OR Bactrim DS 1-2 tabs PO q12h
• Discuss with ID if organisms isolated from recent cultures are resistant to preferred options above. • Discuss dose adjustments with ID if significant renal dysfunction. Use higher doses for patients with higher body weights and/or organisms with elevated MICs. • Usual duration is 7 to 14 days depending on clinical response.	

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Table 3. Preferred IV/PO antibiotics for moderate to severe DFI +/- osteomyelitis

Probability of resistant gram-negative bacteria involvement can be estimated using the following scoring system:

Chronic, deep wound that is older than 30 days	+1
Chronic, deep wound that is older than 90 days	+1
Received a >=5 day course of a beta-lactam/beta-lactamase inhibitor, cephalosporin, carbapenem, bactrim, or fluoroquinolone in the past 90 days	+1
Significant water exposure	+4
CKD or ESRD	+1
PVD	+1
Resident of long-term care facility	+1
TOTAL SCORE	/ 10

